

1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Double-Blind, Placebo-Controlled Study of Mevrometostat (PF-06821497) With Enzalutamide in Metastatic Castration-Sensitive Prostate Cancer (MEVPRO-3)
Brief Study Title: This Study Will Explore Whether a Combination of the Investigational Drug Mevrometostat (PF-06821497) and Enzalutamide Will Work Better Than Taking Enzalutamide Alone in Participants With mCSPC Who Are ARPI naïve. **Short Title/Acronym:** MEVPRO-3 **Protocol Number:** C2321008 **NCT Number:** NCT07028853 **Trial Status:** RECRUITING **Sponsor/Company Name:** Pfizer **Investigational Product:** Mevrometostat (PF-06821497) and Enzalutamide **Phase of Development:** Phase 3 (PHASE3) **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether the combination of the investigational drug mevrometostat and enzalutamide improves radiographic progression-free survival (rPFS) compared to placebo combined with enzalutamide in ARPI-naïve participants with mCSPC. **Secondary Objectives:** To evaluate overall survival (OS), objective response in measurable soft tissue disease, duration of response (DoR), safety, and tolerability of the combination therapy.

2.2 Background & Rationale Enhancer of zeste homolog 2 (EZH2) enzymatic activity plays a crucial role in prostate cancer progression. Mevrometostat, an oral potent and selective EZH2 inhibitor, may synergize with androgen receptor pathway inhibitors (ARPIs) to delay ARPI resistance. In a previous phase I study, mevrometostat combined with enzalutamide showed promising antitumor activity and a manageable safety profile in heavily pre-treated patients with metastatic castration-resistant prostate cancer (mCRPC). This study evaluates the combination earlier in the disease course for patients with metastatic castration-sensitive prostate cancer (mCSPC) who have not yet received systemic chemotherapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1, Stratified (by low vs. high disease volume and de novo vs. relapsed mCSPC)

3.2 Planned Enrollment & Duration Target Enrollment: 1000 participants **Number of Sites:** Global, multicenter **Estimated Study Start:** 28-Sep-2025 **Estimated Primary Completion:** 12-Sep-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male participants aged ≥ 18 years (or the minimum age of consent in accordance with local regulations) at screening. 2. Histologically or cytologically confirmed adenocarcinoma of the prostate without small cell features. 3. Metastatic prostate cancer documented by a positive bone scan (for bone disease) or metastatic lesion(s) on CT or MRI (for soft tissue/visceral disease). 4. Resolution of acute effects of any prior therapy to either baseline severity or CTCAE Grade ≤ 1 (except for AEs which do not constitute a safety risk in the investigator's judgement). 5. Participants must have ECOG Performance Status of 0 or 1.

4.2 Exclusion Criteria 1. Known or suspected brain metastasis or active leptomeningeal disease. 2. Clinically significant cardiovascular disease or inadequate organ function. 3. Prior systemic therapy use in the metastatic setting (must be treatment naïve), with the exception of up to 3 months of androgen deprivation therapy (ADT) and first-generation antiandrogen agents. 4. Any medical or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior or laboratory abnormality that may increase the risk of study participation or make the participant inappropriate for the study. 5. Previous administration with an investigational product (drug or vaccine) within 30 days. 6. Current use or anticipated need for drugs that are known strong CYP3A4/5 inhibitors and inducers (with the exception of enzalutamide as part of this study).

5. Intervention & Treatment Plan

5.1 Investigational Regimen & Drug Information Dosage/Frequency: Arm A: Mevrometostat (875 mg BID with food) + Enzalutamide (160 mg QD); Arm B: Placebo + Enzalutamide (160 mg QD). **Route of Administration:** Oral **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent. **Reference Drug (Enzalutamide):** * **Trade Name(s):** Xtandi * **ATC Code:** L02BB04 * **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/xtandi-epar-product-information_en.pdf

5.2 Concomitant & Prohibited Therapy Permitted: Ongoing ADT (chemical or surgical) to maintain castrate levels; prior palliative radiotherapy or surgery is allowed. **Prohibited:** Radical prostatectomy, prostate radiotherapy, or metastasis-directed therapy for mCSPC; strong CYP3A4/5 inhibitors and inducers; cytotoxic chemotherapy.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Scans (CT/MRI/Bone), Organ Function Labs
Baseline	Day 0	Randomization, First Dose
Interim	Scheduled per Protocol	Safety Labs, Efficacy Assessment (Scans), AE monitoring
End of Study	At Progression	Final Vital Signs, Treatment Discontinuation, Transition to Long-Term Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Radiographic Progression-Free Survival (rPFS). Time from randomization to radiographic progression or death due to any cause, whichever occurs first. **Measurement Method:** Assessed by Blinded Independent Central Review (BICR) using RECIST v1.1 criteria (for soft tissue disease) and PCWG3 criteria (for bone disease).

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS) 2. Objective response in measurable soft tissue disease 3. Duration of Response (DoR) in measurable soft tissue disease

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection via onsite visits and laboratory screening for organ function and toxicities. **Serious Adverse Events (SAEs):** Standard reporting timelines per sponsor and regulatory protocols (typically within 24 hours of site awareness). **Data Monitoring Committee (DMC):** Independent safety review committee designated by the sponsor.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses; Safety Set for all participants receiving at least one dose of study medication. **Sample Size Justification:** Target enrollment of $N = 1000$ to ensure sufficient power to detect a statistically significant improvement in rPFS between the intervention and control arms. **Handling of Missing Data:** Right-censoring methods applied for time-to-event endpoints (rPFS and OS) utilizing Kaplan-Meier estimates.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Scheduled onsite and remote clinical monitoring visits governed by the sponsor. **Audit Trail:** Robust centralized data collection with eCRF locking and Blinded Independent Central Review (BICR) for primary imaging endpoints.

11. Study Identifiers & Governance

Primary ID: C2321008 **NCT Number:** NCT07028853 **Posting ID:** 201150341975708574 **Sponsor/Lead Organization:** Pfizer **Company:** Pfizer **Study Title:** MEVPRO-3

12. Clinical Framework & Ontologies

Disease Areas: Metastatic Castration Sensitive Prostate Cancer (mCSPC), Hormone Sensitive Prostate Cancer, Prostate Cancer, Cancer of the Prostate **Preferred UMLS Name:** Metastatic castration-sensitive prostate cancer **Semantic Classification / Terminology:** * **Name:** Metastatic * **Preferred Name:** Metastatic to * **Semantic Type:** Functional Concept * **Definition:** A term referring to the clinical or pathologic observation of a tumor extension from its original site of growth to another anatomic site. * **MeSH Code:** D009369 * **SNOMED ID:** 263650009 * **SNOMED Hierarchy:** 400177003 | Malignant neoplasm (disorder) | Malignant neoplasm (disorder)

Optimization Target: Delaying ARPI resistance and improving Radiographic Progression-Free Survival (rPFS)

13. Study Procedures & Methodology

Management Pathway: Standard oncology pathway for mCSPC + double-blind investigational treatment **Education Protocol (HCP):** Protocol-specific dosing training (Mevrometostat BID with food) and imaging criteria (RECIST v1.1 / PCWG3) **Education Protocol (Patient):** Informed consent, strict dosing adherence guidelines, and AE self-reporting **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all primary imaging evaluations

14. Observation & Follow-Up Schedule

Total Duration: Treatment Phase followed by Safety Follow-up and Long-Term Follow-up (Study completion estimated Dec 2034) **Onsite Visit Cadence:** Standard oncology visit intervals per study

protocol **Remote Monitoring Frequency:** Survival follow-up at regular intervals post-progression **Checklist Review Interval:** Regular tumor imaging assessment intervals (CT/MRI and bone scans)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	07-Apr-2026				
Clinical Operations Lead	[Name]	_____	07-Apr-2026		Lead		
Statistician	[Name]	_____	07-Apr-2026				